

Study drug will be supplied by the sponsor. The admixture solutions needed for delivery of the lyophilized drug product for SC administration will be sourced locally, or may be supplied by the sponsor, as necessary. Detailed information about the drug product and dose preparation is provided in the pharmacy manual.

8.2. Drug Administration

Patients will be given a dose no greater than 800 mg SC QW (± 1 day) over the treatment period.

The dose and dosing interval of REGN3918 will be finalized based on the emerging data from cohort A of the parent study R3918-PNH-1852, in active PNH patients who have never received a C5 inhibitor, or have not received complement inhibitor therapy in the past 6 months and require treatment with a C5 inhibitor. It is anticipated that most or nearly all patients will continue their dose and dosing interval from the parent study. However, some patients may or may not require an adjustment to their dosage regimen if they enter the current study before the decision on the final dose/dosing interval is made based on the R3918-PNH-1852 study.

The location and administration options for SC route of administration will depend on the preference of the investigator and patient (eg, abdomen, thigh, or upper arm), the availability of clinical supply, and the home healthcare visiting professional. Clinic visits for SC administration may or may not be needed.

If self-administration/administration by patient/designated person is allowed locally, then sufficient injection training at the scheduled injection with REGN3918 will be provided. After training, observation of self-administration/administration by patient/designated person will be conducted by the clinical site personnel or visiting healthcare professional. Once this observation is considered satisfactory, then the study drug can be subsequently administered independently by the patient/designated person for the remainder of the study.

In addition, a patient diary will be provided prior to initiation of self-administration. The diary should be completed upon each the study drug administration. A study drug kit will be dispensed at clinical site visit, using a direct-to-patient (DTP) service provider, or transported by a healthcare professional, as applicable.

Detailed information about the study drug administration is provided in the pharmacy manual.

8.3. Transfusion Algorithm

Transfusions with RBCs during the study should proceed according to the following predefined criteria that will trigger a transfusion; however, the actual number of units to be transfused is at the discretion of the investigator:

- Transfuse with RBC(s) if the post-baseline* hemoglobin level is < 9 g/dL with symptoms resulting from anemia, or
- Transfuse with RBC(s) if the post-baseline* hemoglobin level is < 7 g/dL.

*Baseline is the baseline of OLE study.